Digi Pharm

R1 POC – Maternity Data Gaps

Cluster R1 Maternity Data Gap Analysis Report

Maternity services

In evaluating the data gaps for maternity services in Cluster 1, we have observed that while there was good coverage of data provided, it is essential to acknowledge that the data is currently utilised in a less-than-optimal manner. The data can be considered developmental, implying that it has the potential for improvement and refinement to better serve the needs of the maternity services. To enhance the effectiveness of data usage, we recommend moving towards a scorecard model, specifically adopting the "Maternity Dashboard: Clinical Performance and Governance Score Card" developed by the Royal College of Obstetricians and Gynaecologists (RCOG). This scorecard can be accessed at https://www.rcog.org.uk/media/rqpbkrte/goodpractice7appendix1.pdf.

The proposed scorecard model incorporates key performance indicators (KPIs) that are crucial for evaluating and monitoring the performance and quality of maternity services. We recommend that the KPIs be categorised into the following three elements as presented in the RCOG scorecard:

1. **Activity:** This category focuses on metrics related to the number and types of services provided, patient demographics, and the volume of maternity care delivered. These KPIs would give insights into the overall workload and utilisation of maternity services.

2. **Workforce:** The second element addresses the performance of the healthcare workforce involved in delivering maternity services. It includes KPIs related to staffing levels, qualifications, training, and retention rates. Proper assessment of the workforce is crucial to ensure high-quality care and patient safety.

3. **Clinical Indications:** This element centers around clinical outcomes and adherence to evidence-based practices. The KPIs under this category would encompass various clinical parameters, such as maternal and neonatal mortality rates, caesarean section rates, rates of interventions, and maternal satisfaction scores.

To further enhance data transparency and facilitate cross-verification and audit purposes, it is vital to indicate the sources of the data. For instance, specifying that

the data comes from DATEX (or any other relevant database) would provide stakeholders with information about the origin and reliability of the data being analysed.

By implementing the scorecard model and categorising KPIs into these three elements, Cluster 1's maternity services can gain comprehensive insights into their performance, identify areas of improvement, and make data-driven decisions to enhance overall maternal healthcare. Moreover, this approach promotes standardisation and enables benchmarking against best practices in the field.

In conclusion, the data gaps identified in Cluster 1's maternity services present an opportunity for improvement. By adopting the scorecard model and utilising the proposed KPIs, the healthcare providers in Cluster 1 can move towards an optimised use of data, leading to better patient outcomes and enhanced maternal care.